

**BUSINESS ASSETS**

VYTALIX — Investor one-pager

Vytalix company-build documentation · reference

INVESTOR_ONE_PAGER

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

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Technology for Life · UK health-technology group · Pre-seed · [Founder name] · [email] · [phone] · [website]

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What Vytalix is

A UK health-technology group with four pillars: **Advisory** and **Consulting** (founder-led services for healthcare organisations, health-tech companies, NGOs and governments), **Health Solutions** (first product **MaternaLink**, a maternal care-coordination and communication platform, in development) and **E-Learning** (planned, year 2). Services fund the company; MaternaLink is the venture story.

The problem

- UK maternity reviews (Ockenden, Kirkup) repeatedly identify communication and coordination failures; MBRRACE-UK reports persistent inequalities in maternal outcomes for Black and Asian women.
- In Nigeria, maternal mortality remains among the highest globally, and continuity of contact between women and facilities is weak, especially outside cities.
- Existing tools are either clinical record systems, consumer apps, or donor-funded programmes; few sit in the coordination layer between women, midwives and facilities in a way that is safe, low-bandwidth and interoperable.

What we are building (PROPOSED)

MaternaLink v1: non-diagnostic, offline-tolerant coordination for antenatal to postnatal care: appointment and contact continuity, two-way messaging with escalation to humans, education content governed by clinicians, and programme dashboards. Designed to stay outside medical-device classification, with a documented intended-use statement, clinical-safety governance and UK GDPR compliance. A clinical-AI risk-scoring concept exists as a **research track** only, gated behind data partnerships, ethics approval and a regulatory pathway.

Where we are (honest)

Have	Building	Next
Founder brief and full company build (strategy, product, finance, brand, website, 300-investor CRM)	Entity incorporation, name clearance, IP agreement with the MaternaLink collaborator (Days 1–14)	First paid advisory engagement (target Day 45)
A prototype dashboard (under audit) and a prior government conversation in Nigeria (Ekiti State)	Advisory board recruitment: obstetric/midwifery, NHS digital, Nigerian health system, regulatory	One UK service-evaluation site and one Nigeria NGO cohort in design (targets Day 90)
Re-based product strategy, PRD, MVP spec, roadmap	Research report: "The State of Digital Maternal Care Coordination — UK and Nigeria" (Week 9)	SEIS/EIS advance assurance (applied for after incorporation; TO VALIDATE)

Business model (ESTIMATE)

Advisory day rates £1,200–£1,800; consulting sprints £15k–£60k; MaternaLink pilots £40k–£150k then per-woman-per-year licences (UK £8–£40; Africa £3–£12 at scale). Three-year Base scenario revenue £1.47m with services 59% of the total; break-even not reached within 36 months in Base; see [/docs/08_FINANCIAL_MODEL.md](#).

The raise (PROPOSED)

Pre-seed £250k–£500k (SEIS/EIS-eligible, TO VALIDATE) to fund a product lead, clinical-safety artefacts, the MVP build and two pilots. Milestones before raising: entity, IP deed, retired clinical claims, first services revenue, one pilot LOI. Later rounds only on evidence: Seed £1.5m–£3m (2028), Series A a 2029–30 decision.

Why now, why us

Policy attention on maternity safety and inequality in the UK; state-level digital-health programmes in Nigeria; low-cost messaging infrastructure; a founder who is building the company in the open with a truth-first operating standard and a disciplined investor process.

Ask

20 minutes to test fit against your mandate. Data room available on request once the entity and IP deed are in place.